



Gavin Newsom, Governor
State of California
Health and Human Services Agency
DEPARTMENT OF MANAGED HEALTH CARE
980 9th Street, Suite 500
Sacramento, CA 95814
Phone: 916-324-8176 | Fax: 916-255-5241
www.DMHC.ca.gov

March 19, 2026

VIA ELECTRONIC MAIL

Karen Choate
Blue Cross of California
1121 L Street, Suite 500
Sacramento, CA 95814

Re: Proposed Contract Termination with Santa Clara Valley Medical Center
Block Transfer Filing No. 2026-HOSP-281
Filed on March 19, 2026

Dear Ms. Karen Choate:

The Department of Managed Health Care (Department) is reviewing the information in the above-referenced Block Transfer Filing (Filing) submitted by **Blue Cross of California** (Plan) for compliance with the requirements of the Knox-Keene Health Care Service Plan Act of 1975, as amended (Cal. Health & Safety Code § 1340 *et seq.*) and the regulations promulgated thereunder (Cal. Code Regs., Tit. 28, § 1000 *et seq.*).

In the event of contract termination, the Filing proposes to remove **Santa Clara Valley Medical Center** (Terminating Hospital) from the Plan's hospital network. The effective date of contract termination between the Terminating Hospital and the Plan (Termination Date) is currently **July 1, 2026**.

Please review the following comments and feel free to contact me if you would like to discuss any of the issues before filing the Plan's response.

1. Please confirm the Plan will utilize the most recently approved eFiling Enrollee Transfer Notice Templates for this Block Transfer Filing.

Plan Response1: The Plan will utilize the most recently approved eFiling Enrollee Transfer Notice Templates for this Block Transfer Filing.

2. Please confirm that the Plan is aware that it cannot transition its enrollees or send Enrollee Transfer Notices without the Department's approval.

Plan Response2: The Plan will not transition its enrollees or send Enrollee Transfer Notices without the Department's approval.

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3. Please confirm whether the hospital termination is part of a health system termination. If yes, please confirm the health system and whether the contract is a stand-alone contract.

Plan Response3: The hospital termination is part of a health system termination; the four hospitals are under a single contract.

4. The pending contract termination impacts 50,000 or more enrollees. Please submit the relevant portions of the Provider Contract(s) that relate to continuity of care and transition of care.

Plan Response4: Below are the Provider Contract's continuity of care and transition of care provisions:

Continuance of Care-Termination. If this Agreement is terminated, Facility shall continue to provide and be compensated for Facility services under the terms of this Agreement to Covered Individuals who are Facility inpatients on the date of the termination until those Covered Individuals are discharged or can be safely transferred to another facility. If this Agreement is terminated for reasons other than the grounds set forth in the "Termination With Cause" section, Facility, at Anthem's sole discretion, shall continue to provide and be compensated for Facility services, as required by law, under the terms of this Agreement to Covered Individuals who at the time of termination are receiving services from Facility for one of the following conditions (as defined in Health and Safety Code Section 1373.96): (1) an acute condition; (2) a serious chronic condition; (3) a pregnancy; (4) a terminal illness; (5) care of a newborn child between birth and age thirty-six (36) months; or (6) performance of a surgery or other procedure that has been authorized by Plan (or the relevant delegated medical group/IPA) as part of a documented course of treatment and has been recommended and documented by Facility to occur within one hundred eighty (180) days of the termination date of this Agreement. For cases involving an acute condition, a terminal illness or a pregnancy, such services will continue through the duration of the acute condition, the terminal illness or the pregnancy, respectively. For cases involving a serious chronic condition, such services will continue until the course of treatment has been completed and arrangements have been made for a safe transfer to another participating Facility as determined by Plan in consultation with Facility, consistent with good professional practice, such period not to exceed twelve (12) months from the termination of this Agreement. For cases involving care of a newborn child, as specified above, such services will continue for a period not to exceed twelve (12) months from the termination of this Agreement.

After the effective date of termination, this Agreement shall remain in effect for the resolution of all matters unresolved as of that date.

5. Please confirm whether the contract termination between the Plan and Santa Clara Valley Medical Center impacts Point of Service (POS) enrollees.

Plan Response5: The contract termination between the Plan and the Terminating Provider impacts Point of Service (POS) enrollees.

6. The Plan indicates that Tertiary or Quaternary Care Services are available at the Terminating Hospital. Please confirm which Alternate Hospital(s) provides each service(s).

Plan Response6: Below are the identified Tertiary Services for the Terminating Hospitals and the alternates.

Terminating Hospital - Tertiary Services	Publicly confirmed alternate hospital(s)
Adult Trauma (Level I)	Washington Hospital
Pediatric Trauma Program	Lucile Packard Children's Hospital
Burn Center (adult & pediatric; ABA verified)	None
NICU (Level IV)	Good Samaritan Hospital (Level III); El Camino Hospital – Los Gatos (Level II); Lucile Packard Children's Hospital (Level IV)
PICU	None
High-risk OB / Maternal– Fetal Medicine	Good Samaritan Hospital
ECMO / ECLS	Lucile Packard Children's Hospital

7. The Plan indicates that Burn Services are available at the Terminating Hospital and indicates all Alternate Hospitals offer some level of burn services. Please note that Burn Services refers to the hospital being recognized as a "Burn Center." Please confirm if any Alternate Hospital(s) are Burn Centers.

Plan Response7: The Alternate Hospital(s) are not recognized "Burn Centers".

8. The Plan indicates that Cancer services are available at the Terminating Hospital. Please specify which Cancer services are available at the Terminating Hospital and confirm which Alternate Hospital(s) provides each service(s).

Plan Response 8: The OSHPD/HCAI annual disclosure dataset shows a positive (1) indicator for the Oncology service line at the Terminating Hospital. For Alternate Hospital(s) where the OSHPD/HCAI dataset did not clearly indicate Oncology, the Plan also reviewed facility websites and applied the same indicator-level approach (oncology services offered: Yes/No) to confirm oncology service availability at El Camino, Good Samaritan, Washington, etc. The Plan relied on OSHPD/HCAI where definitive and used website review as supplemental confirmation to support service-line level oncology availability, recognizing that "Oncology" is a broad category and that website content is not standardized or comprehensive.

9. The Plan's PPO line of business is licensed by the Department. Please confirm the following:
- a. The Plan will send an Enrollee Transfer Notice approved by the Department to all PPO enrollees with a 15 mile radius of the Terminating Hospital.

Plan Response9: The Plan will send Department-approved Enrollee Transfer Notices to all PPO enrollees residing within a 15-mile radius of the Terminating Hospital.

10. For each affected provider group that primarily utilizes the Terminating Hospital, please summarize or confirm the following:
- a. The ability of the provider group to redirect enrollees to the Alternate Hospitals:
 - i. Whether there is any specialty for which all of the associated specialists have admitting privileges **only** at the Terminating Hospital and not at any of the Alternate Hospitals,
 - 1. If yes, please identify the specialty and explain how Plan enrollees assigned to the provider group will receive care and services for that specialty following contract termination.
 - ii. If the provider group does not have any primary care physicians or specialists with admitting privileges to one or more Alternate Hospitals, please explain how its assigned enrollees will be admitted to the Alternate Hospital(s) (e.g. via hospitalists, Plan admission process, etc.).
 - 1. If hospitalists are utilized, please indicate:
 - a. The name of the group,
 - b. The number of associated hospitalists, and
 - c. If the hospitalists can admit enrollees for any medically necessary services.

Plan Response10: Provider groups have the full scope of specialty services required by the Plan's contract with its enrollees. For credentialing and contracting purposes, each participating provider group contracted with the Plan must have a full array of specialists, including primary and high-volume specialties, available or accessible to serve our members. The provider groups meet this requirement by either employing and/or contracting with sub-specialists when necessary. No primary or sub-specialties were identified as lacking. If, for some reason, when an In-Network physician or other health care professional specialty is not available or accessible to an enrollee, the enrollee may request an Out-of-Network referral. Furthermore, providers shall arrange for the provision of specialty services from specialists outside the provider's contracted network if unavailable within the network, when medically necessary for the enrollee's condition. Enrollee costs for medically necessary referrals for Out-of-Network providers will not exceed In-Network co-payments, co-insurance, and deductibles.

11. On the Block Transfer Filing form, in the Filing Info section, the Plan indicated there were 11 bed days utilized and 6 admits at the Terminating Hospital for the most recently completed Calendar Year. However, the Plan indicated in it's Narrative that **Santa Clara IPA** utilized 3 bed days, and 2 non-ER admits. Please confirm the following:

- The total number of bed days used, and
- The number of non-ER admits

Plan Response11: The filing submission included a Bed Days & Admit Grid under the documents tab, also provider below. The asterisk (*) denotes the only primarily admitting medical group (utilized 3 bed days / 2 non-ER admits):

SANTA CLARA VALLEY MEDICAL CENTER		
HMO Medical Group	Prior Year Non-ER Bed Days	Prior Year Non-ER Admits
Santa Clara IPA *	3	2
Physicians Medical Group of San Jose	8	4
HMO Sub-Totals	11	6
<i>*Primarily admitting medical group on record.</i>		

PPO Enrollees	Prior Year Non-ER Bed Days	Prior Year Non-ER Admits
	449	48
Grand total for HMO & PPO	460	54

12. For each Alternate Hospital, please summarize or confirm the following:

- The Plan contacted the hospital regarding the Block Transfer,
- Any concerns expressed by the hospital regarding its ability to accommodate the hospital-based needs of enrollees redirected by the Block Transfer,
- Whether the Plan has sent or received a notice of contract termination from the Alternate Hospital, and if so, whether that contract is set to terminate within the next six months.

Plan Response12: The Plan outreaches to the hospitals with occupancy rates at 80% or higher and documented any concerns and capacity response(s) in the filing portal. For hospitals with occupancy rates <80%, justification is not provided as to the sufficiency of the Alternate Hospital's capacity to serve additional plan enrollees per §1300.67.1.3(2)(L): *"Based upon the data made public on the Office of Statewide Health Planning and Development's website, for each of the proposed Alternate Hospitals, provide the available bed occupancy rate for the most recently completed calendar year. If the rate is at 80% or higher, please provide justification as to the sufficiency of the Alternate Hospital's capacity to serve additional plan enrollees."* All alternate hospitals are <80% capacity. The Plan has **NOT** sent or received notice of contract termination from the Alternate Hospitals.

13. The Plan included Lucile Packard Children's Hospital Stanford and Washington Hospital-Fremont as Alternate Hospitals in this filing; however, these hospitals were not included as Alternate Hospitals for Pathway EPO Exchange Network. Please confirm whether Lucile Packard Children's Hospital Stanford and Washington Hospital-Fremont are Alternate Hospitals for Pathway EPO Exchange Network.

Plan Response13: Alternates-Lucile Packard Children's Hospital Stanford and Washington Hospital-Fremont are not included for Pathway EPO Exchange because they are not in the Pathway EPO Exchange Network.

14. Please confirm whether the HMO providers that routinely utilize the Terminating Hospital have adequate admitting privileges for PCPs and specialists to at least one of the identified Alternate Hospitals containing all services that will be lost should the contract ultimately terminate.

Plan Response 14: HMO providers that routinely utilize the Terminating Hospital have adequate admitting privileges for PCPs and specialists to at least one of the identified Alternate Hospitals containing all services.

15. In addition to the in-patient services comparison on the Block Transfer portal, please indicate any of the following services that are available at the Terminating Hospital, but not available at the Alternate Hospitals:
- a. Any out-patient or in-patient services

If any services are not available at the Alternate Hospitals, please explain where affected enrollees will receive these services following contract termination.

Plan Response15: The Plan does not anticipate any problems with the ability of the alternate hospitals to provide the same out-patient and/or in-patient services available at the terminating facility. If there is a service identified that is a covered benefit and only available at the terminating hospital the Plan will make every effort to ensure services are provided at the terminating hospital. When a covered service is not available or accessible at a participating network hospital or specialty facility in the Plan's network members will be allowed to receive that service at a non-participating hospital as appropriate; reimbursement at in-network benefit level.

16. Please confirm the medical groups that do not utilize the Terminating Hospital as a primary hospital, but have admitted enrollees for the most recently completed calendar year, are able to refer their enrollees to other hospitals within the Plan's network for services comparable to those provided by the Terminating Hospital.

Plan Response16: The medical groups that do not utilize the Terminating Hospital as a primary hospital, but have admitted enrollees for the most recently completed calendar year, are able to refer their enrollees to other hospitals within the Plan's network for services comparable to those provided by the Terminating Hospital.

17. Please identify the number of enrollees who have a procedure scheduled and authorized by the Plan at the Terminating Hospital post-termination.

Plan Response17: To date, there are no enrollees who have a procedure scheduled and authorized by the Plan at the Terminating post-termination.

18. Please affirm that the Alternate Hospitals for the Plan's Narrow Networks meet the Knox-Keene Act's geographic access standards and/or Department approved alternative access standards.

Plan Response18: The Plan affirms that Alternate Hospitals for the Plan's Narrow Networks meet the Knox-Keene Act's geographic access standards and/or Department approved alternative access standards.

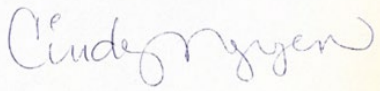
19. Please confirm whether the provider group(s) in this Block Transfer Filing are facing potential contract termination(s) within six months. In addition, if the Plan is required to file a Block Transfer Filing for any of the provider group(s), please provide the name of the provider group and the filing number of each Block Transfer Filing.

Plan Response19: The provider group(s) in this Block Transfer Filing are not facing potential contract termination(s) within six months.

Please submit the Plan's response by **March 26, 2026**, via the Block Transfer portal as a "Response to Comment Letter."

Please contact me if there are any questions regarding the above.

Sincerely,

A handwritten signature in blue ink that reads "Cindy Nguyen". The signature is written in a cursive, flowing style.

Cindy Nguyen
Analyst II
Office of Plan Monitoring